

1. Administrative & Study Identification

Full Study Title: A Phase 3, Randomized, Open-label Study of Relatlimab-nivolumab Fixed-dose Combination Versus Regorafenib or Trifluridine + Tipiracil (TAS-102) for Participants With Later-lines of Metastatic Colorectal Cancer **Short Title/Acronym:** RELATIVITY-123 (A Study of Nivolumab-relatlimab Fixed-dose Combination Versus Regorafenib or TAS-102 in Participants With Later-lines of Metastatic Colorectal Cancer) **Protocol Number:** CA224-123 **NCT Number:** NCT05328908 **Posting ID:** 169672264504000351 **Sponsor Name:** Bristol Myers Squibb (Company: Bristol Myers Squibb) **Investigational Product:** Relatlimab-nivolumab fixed-dose combination (Opdualag; also referred to as BMS-986213); Active Comparators: Regorafenib (Stivarga) or Trifluridine/tipiracil (TAS-102; Lonsurf) **Phase of Development:** Phase III (PHASE3) **Trial Status:** TERMINATED **Medical Monitor:** Bristol Myers Squibb Global Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate Overall Survival (OS) in randomized patients with a PD-L1 combined positive score (CPS) ≥ 1 and OS in all randomized patients. **Secondary Objectives:** To evaluate Objective Response Rate (ORR) by Blinded Independent Central Review (BICR) per RECIST v1.1 in patients with PD-L1 CPS ≥ 1 and in all patients, Progression-Free Survival (PFS) by BICR per RECIST v1.1, and Duration of Response (DOR) by BICR.

2.2 Background & Rationale Metastatic colorectal cancer (mCRC) remains a challenging disease to treat, particularly in later lines of therapy where standard treatments (regorafenib or TAS-102) yield low response rates and limited overall survival. While patients with MSI-H/dMMR tumors have seen significant benefits from immunotherapy, those with microsatellite stable (MSS) or non-MSI-H/dMMR tumors have historically experienced limited efficacy from these agents. This study aimed to determine if the dual inhibition of LAG-3 and PD-1 using the fixed-dose combination of relatlimab and nivolumab could boost immune surveillance and provide meaningful clinical benefit in this high-unmet-need MSS mCRC population. *(Note: The study was discontinued/terminated early due to futility following an independent data monitoring committee analysis).*

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Active Comparator (Investigator's choice of regorafenib or TAS-102) **Blinding:** Open-label **Randomization:** 1:1

3.2 Planned Enrollment & Duration Target N\$: 770 adult patients
Number of Sites: Multicenter, global **Estimated Study Start:** 2022
Estimated Primary Completion: Trial terminated early due to futility (announced January 2024).

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age ≥ 18 years. 2. Histologically confirmed previously treated colorectal cancer with adenocarcinoma histology and metastatic or recurrent unresectable disease (non-MSI-H/dMMR). 3. Must have: i) progressed during or within approximately 3 months following the last administration of approved standard therapies (at least 1, but not more than 4 prior lines of therapies in the metastatic setting), which must include a fluoropyrimidine, oxaliplatin, irinotecan, an anti-VEGF therapy, and anti-EGFR therapy (if RAS wild-type), if available in the respective country, OR; ii) been intolerant to prior systemic chemotherapy regimens if there is documented evidence of clinically significant intolerance despite adequate supportive measures. 4. Must have sufficient tumor tissue & evaluable PD-L1 expression to meet the study requirements. 5. ECOG performance status of ≤ 1 and measurable disease per RECIST v1.1. Participants with lesions in a previously irradiated field as the sole site of measurable disease will be permitted to enroll provided the lesion(s) have demonstrated clear progression and can be measured accurately.

4.2 Exclusion Criteria 1. Confirmed tumor microsatellite instable high/deficient mismatch repair (MSI-H/dMMR) status as per local standard testing. 2. Prior treatment with either an immunotherapy or with regorafenib or with TAS-102. 3. Untreated central nervous system (CNS) metastases (participants are eligible if CNS metastases have been treated and participants have neurologically returned to baseline, except for residual signs or symptoms related to the CNS treatment). 4. History of refractory hypertension not controlled with anti-hypertensive therapy, myocarditis (regardless of etiology), uncontrolled arrhythmias, acute coronary syndrome within 6 months prior to dosing, Class II congestive heart failure (as per the New York Heart Association Functional Classification), interstitial lung disease/pneumonitis or an active, known or suspected autoimmune disease.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: * Arm A

(Investigational): Relatlimab-nivolumab fixed-dose combination (BMS-986213) administered intravenously. * **Arm B (Control):**

Investigator's choice of Regorafenib (Drug Name: regorafenib; Trade Name: Stivarga; ATC Code: L01EX05; [EMA Reference](#)) (oral) or TAS-102 (oral) at standard approved dosing. **Route of**

Administration: Intravenous (IV) for Arm A; Oral for Arm B.

Duration of Treatment: Until disease progression, unacceptable toxicity, withdrawal of consent, or study termination.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care for managing disease-related symptoms and adverse events. **Prohibited:** Concurrent systemic anticancer therapy, concurrent investigational agents, or live vaccines during treatment.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Tumor tissue/PD-L1 evaluation, Baseline Imaging
Baseline	Day 0	Randomization, First Dose
Interim	Varies by Cycle	Safety Labs, Treatment Administration, PK/ADA draws, Efficacy Assessment (Tumor Imaging every 8-12 weeks)
End of Study	Variable	Final Vital Signs, Exit Interview, IP Accountability, Survival Follow-up

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Overall Survival (OS) evaluated in randomized participants with programmed death-ligand 1 (PD-L1) combined positive score (CPS) ≥ 1 and OS in all randomized participants. **Measurement Method:** Time from randomization to death from any cause, assessed via Kaplan-Meier methodology.

7.2 Secondary & Exploratory Endpoints * Objective Response Rate (ORR) assessed by Blinded Independent Central Review (BICR) per RECIST v1.1 in participants with PD-L1 CPS ≥ 1 and all participants. * Progression-Free Survival (PFS) assessed by BICR per RECIST v1.1 in participants with PD-L1 CPS ≥ 1 . * Duration of Response (DOR) assessed by BICR.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring for the incidence of AEs, immune-mediated AEs, and AEs leading to discontinuation. **Serious Adverse Events (SAEs):** Reported standardly to the sponsor and regulatory authorities within 24

hours. **Data Monitoring Committee (DMC):** An independent Data Monitoring Committee (iDMC) oversaw the trial. Based on a planned analysis, the iDMC recommended stopping the trial for futility, though no new safety signals were identified.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for primary efficacy outcomes; Safety Set for all subjects who received at least one dose of study medication. **Sample Size Justification:** A targeted enrollment of 770 patients was planned to provide adequate statistical power to detect a meaningful improvement in Overall Survival. **Handling of Missing Data:** Time-to-event endpoints (OS, PFS) utilized standard Kaplan-Meier estimates and censoring rules for patients without events or lost to follow-up at the time of data cutoff.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study was conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Routine onsite and remote monitoring of trial sites to ensure data integrity and protocol adherence. **Audit Trail:** Standard electronic Case Report Form (eCRF) locking and independent blinding of radiological reads (BICR).

11. Study Identifiers & Governance

Primary ID: CA224-123 **Registry Number:** NCT05328908 **Posting ID:** 169672264504000351 **Sponsor/Lead Organization:** Bristol Myers Squibb **Study Title:** A Study of Nivolumab-relatlimab Fixed-dose Combination Versus Regorafenib or TAS-102 in Participants With Later-lines of Metastatic Colorectal Cancer **Official Regulatory Title:** A Phase 3, Randomized, Open-label Study of Relatlimab-nivolumab Fixed-dose Combination Versus Regorafenib or Trifluridine + Tipiracil (TAS-102) for Participants With Later-lines of Metastatic Colorectal Cancer

12. Clinical Framework (Oncology Specific)

Primary Condition: Metastatic Colorectal Cancer (Preferred UMLS Name: Metastatic Colorectal Cancer) **Disease Areas:** Colorectal Neoplasms **Terminology Codes:** MeSH: D015179 | SNOMED CT: 126837005 | HPO: HP:0100834 **Diagnosis Threshold:** Microsatellite Stable (MSS) or non-MSI-H/dMMR **Medical Methodology:** Immune Checkpoint Inhibition (LAG-3/PD-1) vs. Standard of Care Chemotherapy/Targeted Therapy **Optimization Target:** Extension of Overall Survival (OS)

13. Study Procedures & Methodology

Management Pathway: Advanced/Late-line oncology clinical pathway for mCRC **Education Protocol (HCP):** Site initiation visits and ongoing training on immune-mediated AE management **Education Protocol (Patient):** Informed consent process, reporting of immune-related side effects **Quality Audit Mechanism:** Blinded Independent Central Review (BICR) for efficacy endpoints (RECIST v1.1)

14. Observation & Follow-Up Schedule

Total Duration: Followed until death or study termination **Onsite Visit Cadence:** Aligned with treatment cycles (e.g., every 2 or 4 weeks depending on the assigned arm) **Remote Monitoring Frequency:** Continuous survival follow-up post-treatment **Checklist Review Interval:** Radiological tumor assessments via CT/MRI every 8 to 12 weeks

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]				